



Patient Detail:

Nauraz Fatima .

Age/Sex :

55 (Y) / F

Registration Location:

Near Jinnah Hospital, Lahore

Registration Date:

12-Jun-2026 20:06

Reference:

HABIB BANK LIMITED (Cash I

Consultant:

Dr Nadeem

Patient Number:

45001-26-19803177

Case Number:

17230-12-06



Note : , , ,

Department of Radiology

Reporting Date/Time: 12-Jun-2026 20:44

Abdominal Pelvis Ultrasound

Liver measures 163 mm in the mid clavicular line (Normal upto 160 mm), and shows **mild fatty echotexture** with smooth contours. No focal lesion. Intra hepatic biliary channels are not dilated.

Portal vein measures 9.6 mm and is patent. **CBD** is not dilated.

Gall Bladder is distended measures 95 x 43 mm and shows echogenic sludge and a large stone in the neck region measuring 30 mm. Another floating stone also noted in the gall bladder measuring 10 mm. **GB wall** is of normal thickness.

Pancreas is normal in size, echotexture and outline in its visualized portion.

Spleen measures 81 mm (normal <120 mm). No mass seen.

Rt. Kidney	111 x 39 mm	Cortical thickness measures 13 mm
Lt. Kidney	102 x 46 mm	Cortical thickness measures 13 mm

Renal outlines are smooth. Normal cortico-medullary demarcation. No calculus, mass or hydronephrosis left kidney.

Right kidney shows mild hydronephroureter due to stone measuring 16 mm in the proximal ureter just beyond the pelviureteric junction. No mass.

Urinary Bladder wall is of normal thickness. No stone or mass seen.

No abdominal mass, ascites or enlarged lymph nodes seen.

Tiny para-umbilical hernia defect, measuring 6.4 mm through which omental fat is herniating out. (Reducible)

No basal pleural effusion seen.

Anteverted uterus	76 x 36 x 43 mm	
Rt. Ovary	25 x 10 mm	
Lt. Ovary	29 x 12 mm	

A fibroid in fundal anterior wall measures 40 x 29 x 38 mm pushing the endometrium posteriorly.

Endometrium is not visualized properly due to the compression by the fibroid.

No adnexal mass or cyst seen. No free fluid in culde sac.

IMPRESSION	Mild fatty liver---Grade I. Right proximal ureteric calculus causing mild hydronephroureter. Cholelithiasis and gall bladder sludge. Tiny para-umbilical hernia. Uterine fibroid.
------------	--

This interpretation is based on available information. Correlation with clinical assessment, previous imaging and laboratory investigations is suggested. The study can be reviewed in case of any clinical query.

Dr. Beenish Javed
M.B.B.S, F.C.P.S
Consultant Radiologist

Dr. Adeel Asghar Malik
MBBS, FCPS, FVIR, FRCR(UK)
Consultant Interventional Radiologist

Dr. Rahila Usman
MBBS, FCPS
Consultant Radiologist

Dr. Furqan Ahmad
MBBS (KE), FCPS Diag Rad. (SKMCH)
Diagnostic Imaging Specialist

Dr. Omar Chughtai
MBBS, MD, FCAP
Diplomate American Board of Anatomic
and Clinical Pathology

Dr. A. S. Chughtai
MBBS, MIAC, M.Phil.,
FCPS, FRC PathConsultant Pathologist